

Physician Clinical Narrative

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

07/17/2026 08:14:00

Patient: DEWITTE, ANNE MRN#: A069250 Acct#: 557808 Sex: Female DOB: 07/16/1952 Age: 74y

Disposition: Transfer to Fairview Park Disposition Decision Time: 16:30 07/17/2026 Departure Time: 17:06 07/17/2026

Measurements

Wt: 67.4 kg

Initial Vital Sign

Measured Time	BP	MAP	HR	RR	O ₂ Sat	ETCO ₂	Temp	Pain	GCS	RTS
08:20 07/17/2026	183/92	122	60	16	96% RA		98.5 F	0	15	

Time Seen: 09:09 07/17/2026.

Arrived- By private vehicle. Historian- patient.

HISTORY OF PRESENT ILLNESS

Chief Complaint: PATIENT COMPLAINS OF ELEVATED BLOOD PRESSURE AFTER TAKING SEIZURE MEDICATION. BLOOD PRESSURE ELEVATED. This started PASS FEW DAYS, DENIES HEADACHE, CHEST PAIN , DYSPNEA and is still present. At its maximum, severity described as moderate. When seen in the E.D., severity described as moderate. No loss of appetite, weight loss, headache, fatigue or muscle aches. Denies sleep problem. No decreased urine output.

Similar symptoms previously. Patient has had similar symptoms several times.

Recent medical care: Not recently seen/assessed.

REVIEW OF SYSTEMS

CONSTITUTIONAL: No fever or chills. **THROAT:** No sore throat. **NOSE:** No sinus drainage or nasal congestion. **CVS:** No chest pain or calf pain. **GI:** No abdominal pain, nausea, black stools or bloody stools. **SKIN:** No skin rash. **MUSCULOSKELETAL:** No back pain. **NEUROLOGICAL:** No headache or blackouts. No difficulty with ambulation. **EYES:** No double vision. **RESPIRATORY:** No cough or difficulty breathing. All other systems reviewed and are negative.

PAST HISTORY

See nurses notes.

Hypertension

Seizure Disorder

Surgeries:

Hysterectomy

Knee Surgery: Body Site RIGHT KNEE

Medications:

carBAMazepine 200 mg tablet: 200 mg every 12 hours .

fosinopril 40 mg tablet: 40 mg once a day .

Allergies:

Keppra: Intolerance. (SPOUSE STATES "SHE WILL BE OUT 3 DAYS" AFTER TAKING 1 DOSE OF KEPPRA)

SOCIAL HISTORY

Former smoker. No alcohol use or drug use. No recent travel. Resides in a house. The patient lives with spouse.

FAMILY HISTORY

Negative.

ADDITIONAL NOTES
The nursing notes have been reviewed.

PHYSICAL EXAM
Vital Signs: Have been reviewed.
Appearance: Alert.
Eyes: No conjunctival findings, scleral icterus or pale conjunctivae.
ENT: Ears normal. Nose normal. Normal ear exam. No nasal discharge, pharyngeal erythema or tonsillar exudate.
Neck: Normal inspection. Neck supple. No meningeal signs, JVD, carotid bruit, lymphadenopathy or thyromegaly.
CVS: Normal heart rate. Heart sounds normal. Rate normal. Rhythm normal. No extra heart sounds. PMI not displaced laterally. No decreased pulses.
Respiratory: No respiratory distress. No respiratory distress. Painless inspiration. Breath sounds normal. Chest nontender. No accessory muscle use, decreased air movement, crackles or wheezes.
Abdomen: No visible injury. Soft. No organomegaly. No mass. Femoral pulses equal.
Back: Normal inspection. No CVA tenderness.
Skin: Cyanosis. Skin warm. Normal skin color.
Extremities: No lower extremity edema.
Neuro: Oriented X 3. No alteration in mental status. No cranial nerve deficit. No motor deficit. No weakness. No sensory deficit.

LABS, X-RAYS, AND EKG
Diagnostic Tests: Diagnostic tests have been ordered, with results reviewed and considered in the medical decision making process.
Laboratory Tests: PATIENT GIVEN HYDRALAZINE 20MG IV FOR BP 202/105
IMPROVED TO A BP 140/76

ADMINISTERED ROCEPHIN 1GM IV FOR UTI AND MAXILLARY SINUSITIS.

Laboratory Tests:

CBC AUTO DIFF	Final	COLL: 07/17/2026 09:45:00 EDT MsgRcvd: 07/17/2026 10:16 EDT		
Lab Test	Result	Reference	Status	Received
CBC AUTO DIFF			Final	07/17/2026 10:16:24 EDT
COMPLETE BLOOD COUNT				
WBC	6.1 K/uL	4.0 - 10.8	Final	07/17/2026 10:16:24 EDT
RBC	4.78 M/uL	3.90 - 5.40	Final	07/17/2026 10:16:24 EDT
HEMOGLOBIN	15.5 g/dL	12.0 - 16.0	Final	07/17/2026 10:16:24 EDT
HEMATOCRIT	43.1 %	35.0 - 47.0	Final	07/17/2026 10:16:24 EDT
MCV	90 fL	80 - 100	Final	07/17/2026 10:16:24 EDT
MCH	32.4 pg	27.0 - 33.0	Final	07/17/2026 10:16:24 EDT
MCHC	36 g/dL	31 - 36	Final	07/17/2026 10:16:24 EDT
RDW	13.4 %	11.5 - 15.5	Final	07/17/2026 10:16:24 EDT

PLATELETS	169 K/uL	139 - 358	Final	07/17/2026 10:16:24 EDT
MPV	7.7 fL	7.4 - 10.4	Final	07/17/2026 10:16:24 EDT
%NEUT	72 %	40 - 77	Final	07/17/2026 10:16:24 EDT
%LYMPH	21 %	14 - 48	Final	07/17/2026 10:16:24 EDT
%MONO	6 %	4 - 11	Final	07/17/2026 10:16:24 EDT
%EOS	1 %	0 - 7	Final	07/17/2026 10:16:24 EDT
%BASO	1 %	0 - 1	Final	07/17/2026 10:16:24 EDT
#NEUT	4.4 K/uL	1.9 - 7.3	Final	07/17/2026 10:16:24 EDT
#LYMPH	1.3 K/uL	0.9 - 3.8	Final	07/17/2026 10:16:24 EDT
#MONO	0.4 K/uL	0.2 - 0.9	Final	07/17/2026 10:16:24 EDT
#EOS	0.0 K/uL	0.0 - 0.5	Final	07/17/2026 10:16:24 EDT
#BASO	0.0 K/uL	0.0 - 0.1	Final	07/17/2026 10:16:24 EDT
MANUAL DIFF	NOT INDICATED		Final	07/17/2026 10:16:24 EDT
RBC MORPH	NOT INDICATED		Final	07/17/2026 10:16:24 EDT
{CD}				
ANALYSIS PERFORMED ON BECKMAN-COULTER DXH900 AS OF AUGUST 15, 2022.				

COMPREHENSIVE METABOLIC PANEL		Final	COLL: 07/17/2026 10:48:00 EDT MsgRcvd: 07/17/2026 11:12 EDT		
Lab Test	Result		Reference	Status	Received
COMPREHENSIVE METABOLIC PANEL				Final	07/17/2026 11:12:31 EDT
COMPREHENSIVE METABOLIC PANEL					
SODIUM	142 mEq/L		135 - 145	Final	07/17/2026 11:12:31 EDT
POTASSIUM	4.5 mEq/L		3.5 - 5.0	Final	07/17/2026 11:12:31 EDT
CHLORIDE	106 mEq/L		98 - 112	Final	07/17/2026 11:12:31 EDT
CO2	24 mEq/L		21 - 31	Final	07/17/2026 11:12:31 EDT

GLUCOSE	105 mg/dL	70 - 110	Final	07/17/2026 11:12:31 EDT
BUN	13 mg/dL	8 - 22	Final	07/17/2026 11:12:31 EDT
CREATININE	0.65 mg/dL	0.50 - 1.20	Final	07/17/2026 11:12:31 EDT
BUN/CREAT	20.00	6.00 - 25.00	Final	07/17/2026 11:12:31 EDT
CALCIUM	10.3 mg/dL	8.5 - 10.5	Final	07/17/2026 11:12:31 EDT
SGPT/ALT	15 IU/L	7 - 52	Final	07/17/2026 11:12:31 EDT
TOTAL BILI	0.4 mg/dL	0.0 - 1.3	Final	07/17/2026 11:12:31 EDT
TOTAL PROTEIN	7.3 g/dL	6.5 - 8.2	Final	07/17/2026 11:12:31 EDT
ALBUMIN	4.1 g/dL	3.2 - 5.5	Final	07/17/2026 11:12:31 EDT
A/G RATIO	1.28	0.80 - 2.00	Final	07/17/2026 11:12:31 EDT
SGOT/AST	24 IU/L	13 - 39	Final	07/17/2026 11:12:31 EDT
ALKALINE PHOS	64 IU/L	42 - 121	Final	07/17/2026 11:12:31 EDT
AGE	74 yrs		Final	07/17/2026 11:12:31 EDT
NON-AA GFR	95 mL/min		Final	07/17/2026 11:12:31 EDT
AFR AMER GFR	115 mL/min		Final	07/17/2026 11:12:31 EDT

REFERENCE RANGE TABLE FOR POPULATION MEAN GFRs FROM NKDEP
AGE (years) AVERAGE GFR
20-29 116 ml/min/1.73m
30-39 107 ml/min/1.73m
40-49 99 ml/min/1.73m
50-59 93 ml/min/1.73m
60-69 85 ml/min/1.73m
70+ 75 ml/min/1.73m
Chronic kidney disease is defined as either kidney damage or GFR less than 60 for at least three months. eGFR should be correlated with patient's clinical status. eGFR should not be used to calculate drug dosage.

DRUG SCREEN URINE/BMH		Final	COLL: 07/17/2026 09:45:00 EDT			MsgRcvd: 07/17/2026 10:23 EDT	
Lab Test	Result		Reference	Status	Received		
DRUG SCREEN URINE/BMH				Final	07/17/2026 10:23:28 EDT		
DRUG SCREEN							

AMPHETAMINES	NEGATIVE		Final	07/17/2026 10:23:28 EDT
BARBITURATES	NEGATIVE		Final	07/17/2026 10:23:28 EDT
BENZO	NEGATIVE		Final	07/17/2026 10:23:28 EDT
COCAINE	NEGATIVE		Final	07/17/2026 10:23:28 EDT
METHADONE	NEGATIVE		Final	07/17/2026 10:23:28 EDT
OPIATES	NEGATIVE		Final	07/17/2026 10:23:28 EDT
PCP	NEGATIVE		Final	07/17/2026 10:23:28 EDT
THC	NEGATIVE		Final	07/17/2026 10:23:28 EDT
OXYCODONE	NEGATIVE		Final	07/17/2026 10:23:28 EDT
URINE CREATININE	44.7		Final	07/17/2026 10:23:28 EDT

\\BLDo\URINE DRUG SCREEN INTERPRETATION\BLDx\

Qualitative results only. A result of "Negative" may mean that the drug is present, but below the cut-off value. A result of "Positive" indicates a presence of the drug above the cut-off value in the sample. Additional tests are necessary for confirmation.

CUT OFF VALUES:

AMPHETAMINE: 1000 ng/mL

BARBITURATE: 200 ng/mL

BENZO: 200 ng/mL

COCAINE: 300 ng/mL

METHADONE: 300 ng/mL

OPIATES: 300 ng/mL

PCP: 25 ng/mL

THC: 50 ng/mL

OXYCODONE: 100 ng/mL

MAGNESIUM	Final	COLL: 07/17/2026 10:48:00 EDT MsgRcvd: 07/17/2026 11:12 EDT		
Lab Test	Result	Reference	Status	Received
MAGNESIUM	2.1 mg/dL	1.9 - 2.7	Final	07/17/2026 11:12:34 EDT

TROPONIN HS HIGH SENSITIVITY	Final	COLL: 07/17/2026 10:48:00 EDT MsgRcvd: 07/17/2026 13:25 EDT		
Lab Test	Result	Reference	Status	Received
TROPONIN HS	2.3 pg/mL	0.0 - 15.0	Final	07/17/2026 13:25:49 EDT

Potential for acute myocardial infarction when correlated with clinical suspicion for acute coronary syndrome.

URINALYSIS WITH REFLEX	Final	COLL: 07/17/2026 09:45:00 EDT MsgRcvd: 07/17/2026 10:32 EDT		
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Lab Test	Result	Reference	Status	Received
URINALYSIS WITH REFLEX			Final	07/17/2026 10:32:56 EDT
URINALYSIS WITH REFLEX				
SPEC SOURCE:	CLEAN CATCH		Final	07/17/2026 10:32:56 EDT
COLOR	COLORLESS	NORMAL: Straw	Final	07/17/2026 10:32:56 EDT
CLARITY	CLEAR	NORMAL: Clear	Final	07/17/2026 10:32:56 EDT
SPEC GRAVITY	1.009	1.005 - 1.020	Final	07/17/2026 10:32:56 EDT
pH	7.5	5.5 - 7.5	Final	07/17/2026 10:32:56 EDT
GLUCOSE	NORMAL	NORMAL: Negative	Final	07/17/2026 10:32:56 EDT
BILIRUBIN	NEGATIVE	NORMAL: Negative	Final	07/17/2026 10:32:56 EDT
KETONE	NEGATIVE	NORMAL: Negative	Final	07/17/2026 10:32:56 EDT
PROTEIN	NEGATIVE	NORMAL: Negative	Final	07/17/2026 10:32:56 EDT
NITRITE	NEGATIVE	NORMAL: Negative	Final	07/17/2026 10:32:56 EDT
BLOOD	TRACE	NORMAL: Negative	Final	07/17/2026 10:32:56 EDT
LEUK EST	250 Abnormal	NORMAL: Negative	Final	07/17/2026 10:32:56 EDT
UROBILINOGEN	NORMAL	0.2 - 1.0 mg/dL	Final	07/17/2026 10:32:56 EDT
MICROSCOPIC	See Below		Final	07/17/2026 10:32:56 EDT
WBC	1 - 3	NORMAL: None Seen	Final	07/17/2026 10:32:56 EDT
RBC	1 - 3	NORMAL: None Seen	Final	07/17/2026 10:32:56 EDT
EPITHELIAL	See Below	NORMAL: None Seen	Final	07/17/2026 10:32:56 EDT
SQUAMOUS EPI	0-5	NORMAL: None Seen	Final	07/17/2026 10:32:56 EDT
Bacteria	Rare/hpf	NORMAL: None Seen	Final	07/17/2026 10:32:56 EDT

TROPONIN HS HIGH SENSITIVITY Final

COLL: 07/17/2026 14:50:00 EDT MsgRcvd: 07/17/2026 15:43 EDT

Lab Test	Result	Reference	Status	Received
TROPONIN HS	28.2 pg/mL Above high normal	0.0 - 15.0	Final	07/17/2026 15:43:34 EDT
Potential for acute myocardial infarction when correlated with clinical suspicion for acute coronary syndrome.				

Diagnostic Study Tests:

CT HEAD WO CONTRAST

Final

MsgRcvd: 07/17/2026 14:22 EDT

BLECKLEY MEMORIAL HOSPITAL
145 E. PEACOCK STREET
COCHRAN, GA. 31014
-----NAME----- NUMBER SEX AGE ADMIT DISC. XRAY# F/C TYPE
DEWITTE ANNE C 557808 F 74 7/17/26 38696 MB E/R
DATE OF BIRTH: 07/16/1952 M/R# A069250 PH#: 229-947-2637 RM ER5
LOCATION: TRANSCRIBED: 07/17/26 14:18
CT HEAD WO CONTRAST 70450 COMPLETED:07/17/26 14:03 NOV 7930
ReasonForStudy: Head Pain
PHYSICIAN: WATKINS DW OATES MICH

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RADIOLOGY REPORT

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EXAM:
CT HEAD WO CONTRAST
HISTORY:
HEAD PAIN NO PRIOR ;
COMPARISON:
No relevant prior studies were available for comparison at the time of interpretation..
TECHNIQUE:
CT images were obtained. Multiplanar reconstructions were created on a separate workstation and used during interpretation. All CT scans at this facility is dose modulation, iterative reconstruction, and/or weight-based dosing as appropriate to reduce radiation to levels as low as reasonably achievable (ALARA). Postprocessing details, radiation dose, and contrast dose (if applicable) are recorded in the patient's medical record.
FINDINGS:
Head:
Acute findings: There is no intracranial hemorrhage. No mass effect. No intra-axial or extra-axial fluid collection. There is no mass. No tentorial, uncal, or tonsillar herniation.
Brain volume and white matter: Brain volume and attenuation is normal for age.
Ventricles: No hydrocephalus
Midline structures: Pituitary gland and corpus callosum are normal.
Posterior fossa and skull base: Cerebellum and posterior fossa are within normal limits. Basal cisterns are not effaced.
Sinuses and mastoids: Acute sinusitis suggested by fluid in the right maxillary sinus
Globes and Orbits: Globes are intact. Bony orbits are intact. Orbital contents are unremarkable.
Skull and soft tissues: No depressed skull fracture. Calvarium appears intact. No scalp injury is identified.
IMPRESSION:
1. Right maxillary sinusitis suggested
2. No acute intracranial abnormality
THIS IS AN ELECTRONICALLY VERIFIED FINAL REPORT
7/17/2026 2:18 PM - Electronically signed by Brent Jordan, MD
Reviewed and Electronically Signed By:
BRENT JORDAN MD

RADIOLOGIST

Date/Time: 07/17/26 14:18

PROGRESS AND PROCEDURES

Critical care performed (30 minutes). Time includes: direct patient care, patient reassessment, interpretation of data (laboratory data, pulse oximetry and chest xrays) and documentation of patient care. Procedures included in critical care time: peripheral IV placement and phlebotomy.

Differential Diagnosis:

Other possible considerations: ELEVATED TROPONIN

HYPERTENSION

URINARY TRACT INFECTION.

Above considerations are based on history, physical exam and past history.

Differential diagnosis was discussed with patient.

MEDICAL DECISION MAKING: MEDICAL COMPLEXITY MODERATE. Pertinent clinical findings include the presentation: ELEVATED BLOOD PRESSURE and history: HISTORY OF HYPERTENSION, SEIZURE DISORDER. Serious conditions are unlikely to be a cause for the patient's findings. The differential diagnosis includes, but is not limited to, (ELEVATED TROPONIN, HEADACHE TIA, CVA). The diagnosis appears to be straight forward. Ordered tests include a CT of the head, a complete blood count and chemistries. IV fluids were given. The pain has resolved. (PATIENT WITH A HISTORY OF HYPERTENSION AND SEIZURE DISORDER, AND SINCE BEING PRESCRIBED TEGRETOL 200MG TWICE DAILY ONSET OF ELEVATED BLOOD PRESSURE THE PAST 2 WEEKS. DENIES CHEST PAIN, DYSPNEA, HEADACHE BLURRED VISION. SERIAL TROPONINS 2.3 AND 28, SERIAL EKG'S INITIAL SINUS BRADYCARDIA, 2ND EKG SINUS RHYTHM RATE 90 WITH NEW ST SEGMENT DEPRESSION INFERIOR LATERALLY).

Disposition: Disposition Decision Time: 16:29 07/17/2026. Patient transferred to Fairview Park. Departure Time: 16:30 07/17/2026. Transferred in stable condition. Decision for transfer based upon need for higher level of care; continuity of care. Benefits, risks and alternatives to transfer explained to patient and spouse. Case discussed with accepting provider DR SARATA AT 1600. Information relayed regarding patient's history and radiology. Disposition Decision Time: 16:29 07/17/2026. Patient transferred. Discharged in stable condition. Discharge decision based on the following: patient's condition is stable; patient's exam is stable; no abnormal test results; multiple repeat evaluations; social support is adequate; clinical impression is consistent with outpatient treatment.

CLINICAL IMPRESSION

ELEVATED TROPONIN

HYPERTENSION

URINARY TRACT INFECTION.

(Electronically signed by Michael Oates, M.D. 07/17/26 17:08:42 EDT)

Disposition History:

Disposition Decision Time: 16:29 07/17/2026. Disposition changed to Transfer. -- 16:29 07/17/2026 Michael Oates, M.D.

Disposition Decision Time: 16:29 07/17/2026. Disposition changed to Transfer to Fairview Park. Departure Time: 16:30 07/17/2026. -- 16:30 07/17/2026 Michael Oates, M.D.

Disposition: (none). -- 16:34 07/17/2026 Lusha Fowler, R.N.

Disposition Decision Time: 16:30 07/17/2026. Disposition changed to Transfer to Fairview Park. -- 16:34 07/17/2026 Lusha Fowler, R.N.

Generated by CorroHealth

Nurse Narrative

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

07/17/2026 08:14:00

Patient: DEWITTE, ANNE	MRN#: A069250	Acct#: 557808	Sex: Female	DOB: 07/16/1952	Age: 74y
Disposition: Transfer to Fairview Park	Disposition Decision Time: 16:30 07/17/2026			Departure Time: 17:06 07/17/2026	

TRIAGE

Triage time: 08:15 07/17/2026. -- 08:28 07/17/26 EDT Lusha Fowler, R.N.

Arrived by private vehicle, and from home. Historian: (spouse and patient). Accompanied by spouse.

Acuity: LEVEL 3.

Chief Complaint: (ELEVATED BP). BLOOD PRESSURE ELEVATED. Checked by patient at home 169 / 112.

08:15 07/17/26. Alert.

This started last night. Onset. (SPOUSE STATES IS FROM DR. EASTERLING PUTTING PT ON CARBAMAZEPINE 5 DAYS AGO). (SPOUSE C/O PT"STAYED IN BED MOST OF LAST 2 DAYS, AND JUST SAT THERE STARED,AND THAT SHE HAS BEEN UNABLE TO COMPLETE SENTENCES SINCE SHE BEGAN CARBAMAZEPINE). -- 08:35 07/17/26 EDT Lusha Fowler, R.N.

08:20 07/17/26. BP: 183/92 (regular cuff) taken on left arm, while sitting. MAP: 122. HR: 60. Regular and normal rate. RR: 16. Regular and unlabored. O2 saturation: 96% on room air. Temperature: 98.5 F. Pain level now 0/10. Glasgow Coma Scale: 15 - eyes open - spontaneous (4); best verbal response - oriented (5); best motor response - obeys commands (6). -- 08:43 07/17/26 EDT Lusha Fowler, R.N.

Measurements:

08:41 07/17/26 Wt: 67.4 kg -- 08:41 07/17/26 EDT Lusha Fowler, R.N.

Medications:

fosinopril 40 mg tablet: 40 mg once a day . -- 08:32 07/17/26 EDT Lusha Fowler, R.N.

carBAMazepine 200 mg tablet: 200 mg every 12 hours . -- 08:32 07/17/26 EDT Lusha Fowler, R.N.

08:15 07/17/26.

Preferred Pharmacy: PUBLIX; BONAIRE. -- 08:35 07/17/26 EDT Lusha Fowler, R.N.

Allergies:

Keppra: Intolerance. (SPOUSE STATES "SHE WILL BE OUT 3 DAYS" AFTER TAKING 1 DOSE OF KEPPRA) -- 08:32 07/17/26 EDT Lusha Fowler, R.N.

Problems:

Hypertension -- 08:32 07/17/26 EDT Lusha Fowler, R.N.

Seizure Disorder -- 08:33 07/17/26 EDT Lusha Fowler, R.N.

Surgeries:

Hysterectomy -- 08:33 07/17/26 EDT Lusha Fowler, R.N.

Knee Surgery. RIGHT KNEE -- 08:33 07/17/26 EDT Lusha Fowler, R.N.

History

08:15 07/17/26.

PAST MEDICAL HX: LNMP: The patient has had a hysterectomy.

Denies current pregnancy.

SOCIAL HX: Former smoker, end date 1976. The patient has not traveled outside the U.S.

Infectious disease exposure: No infectious disease exposure.

ABUSE ASSESSMENT: The patient answered "yes" to the question(s) "Do you feel safe in your home?" and "no" to the question(s) "Are you afraid to go home?". No suspicion of abuse.

SELF HARM ASSESSMENT: Self harm assessment was performed. The patient answered "no" to the question(s) "Are you here because you tried to hurt yourself?".

NUTRITIONAL RISK ASSESSMENT: The nutritional risk assessment revealed no deficiencies.

FUNCTIONAL ASSESSMENT: Functional assessment: no impairments noted.

LEARNING NEEDS ASSESSMENT: The learning needs assessment revealed no barriers.

SKIN INTEGRITY ASSESSMENT: Skin integrity risk assessment completed. No skin integrity risk identified. -- 08:35 07/17/26 EDT Lusha Fowler, R.N.

Interventions

08:15 07/17/26. Identification band on patient. To treatment room. -- 08:35 07/17/26 EDT Lusha Fowler, R.N.

08:40 07/17/26. Advanced care plan discussed with patient and family. Patient does not have advanced directive. --

08:46 07/17/26 EDT Lusha Fowler, R.N.

PHYSICAL ASSESSMENT

08:15 07/17/26. Ambulatory to room.

GENERAL / NEURO / PSYCH: Alert. Oriented X 4. Slow gait. Ambulates using a cane.

HEENT: Pupils equal, round and reactive to light. No facial asymmetry noted.

RESPIRATORY: Respirations not labored. Breath sounds within normal limits.

CVS: Capillary refill less than 2 seconds. Pulses within normal limits.

GI / GU: Abdomen soft.

SKIN: Skin intact. Skin is warm and dry. Normal skin turgor. -- 08:47 07/17/26 EDT Lusha Fowler, R.N.

NURSING PROGRESS NOTES

08:15 07/17/26. Patient identifiers checked. Call light placed in reach. Bed placed in lowest position. Brakes of bed on. -- 08:35 07/17/26 EDT Lusha Fowler, R.N.

08:25 07/17/26. Patient ready for evaluation- ED physician notified. (UNABLE TO REACH-BY CALLING DR'S ROOM-NO CELL NUMBER AVAILABLE). -- 08:36 07/17/26 EDT Lusha Fowler, R.N.

09:30 07/17/26.

12-LEAD EKG: EKG time: (09:21 07/17/2026). 12-Lead EKG was ordered and performed by a respiratory therapist (09:25 07/17/2026). -- 10:32 07/17/26 EDT Lusha Fowler, R.N.

09:45 07/17/26. Checked patient name and birthdate: patient confirmed. Blood samples drawn from the right antecubital space peripheral IV site with 20g by me ; labeled in presence of the patient and sent to lab: green, purple, blue and yellow top. Attempted x1. -- 10:29 07/17/26 EDT Lusha Fowler, R.N.

09:45 07/17/26. Site #1 started in the right antecubital space with a 20g needle with aseptic technique and good blood return; 1 attempt. Blood drawn: green, purple, blue and yellow tube(s). Saline lock flushed with 10 mL saline. -- 09:46 07/17/26 EDT Lusha Fowler, R.N.

09:49 07/17/26. IV NS 500 mL started in bag#1 500 mL at 250 mL/hr via Site# 1. Allergies verified and confirmed 5 rights. Via IV pump. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly pre-medication administration. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 09:49 07/17/26 EDT Lusha Fowler, R.N.

10:00 07/17/26. Checked patient name and birthdate urine collected with return of yellow-colored clear urine; odor is normal; sample sent to lab for urinalysis, culture and drug screen. Specimen labeled in the presence of the patient. -- 10:33 07/17/26 EDT Lusha Fowler, R.N.

10:48 07/17/26. Checked patient name and birthdate. Blood samples drawn from the left antecubital space with Vacutainer and 21g needle by lab ; labeled in presence of the patient and sent to lab. Attempted x1. (REDRAW). -- 10:51 07/17/26 EDT Lusha Fowler, R.N.

10:55 07/17/26. IV NS: Medication Discontinued. bag #1 infused. Total amount infused: 500 mL. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly post-medication administration. -- 11:06 07/17/26 EDT Lusha Fowler, R.N.

11:00 07/17/26. BP: 174/92 taken on right arm, while sitting. MAP: 119. HR: 56. Regular and bradycardic. RR: 16. Regular and unlabored. O2 saturation: 100% on room air. Glasgow Coma Scale: 15 - eyes open - spontaneous (4); best verbal response - oriented (5); best motor response - obeys commands (6). -- 11:07 07/17/26 EDT Lusha Fowler, R.N.

11:28 07/17/26. Tylenol PO 650 mg given. Allergies verified and confirmed 5 rights. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 11:28 07/17/26 EDT Lusha Fowler, R.N.

11:29 07/17/26. Rocephin IVPB 1g/100ml NS 1 g started at 200 mL/hr diluted in sodium chloride IVPB 0.9 % (minibag plus) 100 mL via Site# 1. Allergies verified and confirmed 5 rights. Via IV pump. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly pre-medication administration. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 11:29 07/17/26 EDT Lusha Fowler, R.N.

12:00 07/17/26. Rocephin IVPB 1g/100ml NS: Medication Discontinued. IV infused. Total amount infused: 100 mL. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly post-medication administration. -- 12:02 07/17/26 EDT Lusha Fowler, R.N.

12:05 07/17/26. HydrALAZINE IVP 20 mg given via Site# 1. Allergies verified and confirmed 5 rights. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly pre-medication administration. IVP given by nurse. Information reviewed with patient and spouse including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 12:05 07/17/26 EDT Lusha Fowler, R.N.

13:00 07/17/26. IV NS Bolus 1000 mL started in bag#1 1000 mL wide open via Site# 1. Allergies verified and confirmed 5 rights. Via IV pump. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed

thoroughly pre-medication administration. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 13:01 07/17/26 EDT Shalie Shiver, L.P.N.

13:55 07/17/26. Patient transported to CT by wheelchair with radiology tech. -- 13:56 07/17/26 EDT Lusha Fowler, R.N.

13:55 07/17/26. (DR.OATES STATES PT WITH H/A-HAS ALREADY HAD TYLENOL.). -- 13:58 07/17/26 EDT Lusha Fowler, R.N.

14:00 07/17/26. IV NS: Medication Discontinued. bag #2. Total amount infused: 1000 mL. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly post-medication administration. -- 14:27 07/17/26 EDT Lusha Fowler, R.N.

14:18 07/17/26. BP: 134/64 (regular cuff) taken on left arm, while lying. MAP: 87. HR: 92. Regular and normal rate. RR: 16. Regular and unlabored. O2 saturation: 98% -- 14:21 07/17/26 EDT Lusha Fowler, R.N.

14:20 07/17/26. (PT DENIES HEADACH AT THIS TIME). -- 14:20 07/17/26 EDT Lusha Fowler, R.N.

14:28 07/17/26.

12-LEAD EKG #2: EKG time: (14:27 07/17/2026). 12-Lead EKG was performed by a respiratory therapist and shown to the ED physician (14:28 07/17/2026). -- 14:44 07/17/26 EDT Shalie Shiver, L.P.N.

14:50 07/17/26. Checked patient name and birthdate: patient confirmed. Blood samples drawn from the left antecubital space with Vacutainer and 22g needle by lab ; labeled in presence of the patient and sent to lab: green top; cardiac enzymes (2nd set). Attempted x1. -- 14:55 07/17/26 EDT Lusha Fowler, R.N.

14:53 07/17/26. Patient ID band checked for patient name and birthdate: patient confirmed. Blood samples drawn from the left antecubital space with Vacutainer 23g by lab per protocol ; labeled in presence of the patient and sent to lab: green top. Attempted x1. -- 14:55 07/17/26 EDT Shalie Shiver, L.P.N.

14:56 07/17/26. KetorOLAC IVP 15 mg given via Site# 1. Allergies verified and confirmed 5 rights. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly pre-medication administration. IVP given by LPN. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 14:56 07/17/26 EDT Shalie Shiver, L.P.N.

15:47 07/17/26. (CALL PLACED TO ONE STEP FOR POSSIBLE TRANSFER TO FAIRVIEW PARK HOSPITAL-SPOKE WITH TRACEY.GAVE PT INFO). -- 15:52 07/17/26 EDT Lusha Fowler, R.N.

15:51 07/17/26. (CALL TRANSFERED TO DR.OATES). -- 15:52 07/17/26 EDT Lusha Fowler, R.N.

15:53 07/17/26. Aspirin PO 325 mg given. Allergies verified and confirmed 5 rights. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 15:54 07/17/26 EDT Lusha Fowler, R.N.

15:56 07/17/26. (PER ONE STEP TRANSFER PT ACCEPTED TO FAIRVIEW PARK HOSPITAL ER BY DR.JEFFERY SARATA.). -- 15:59 07/17/26 EDT Lusha Fowler, R.N.

16:02 07/17/26. (CALL PLACED TO HEARTLAND DISPATCH-SPOKE WITH KAYLA. STATES ETA 45MIN TO HR). -- 16:02 07/17/26 EDT Lusha Fowler, R.N.

16:51 07/17/26. Portable CXR performed. -- 16:51 07/17/26 EDT Lusha Fowler, R.N.

17:05 07/17/26. Site #1 in place upon transfer; patent, no pain, no signs of infection and no signs of infiltration. -- 17:06 07/17/26 EDT Lusha Fowler, R.N.

DISPOSITION / DISCHARGE

16:10 07/17/26. Report was given to a nurse via a phone call. Report included information regarding patient's care and allergies and current and abnormal vital signs. Report included treatment information regarding type and amount of IV fluids total volume infused. All questions were answered. Report was acknowledged (ANTHONY,RN). -- 16:12 07/17/26 EDT Lusha Fowler, R.N.

17:00 07/17/26. BP: 175/83 (regular cuff) taken on left arm, while lying. MAP: 114. HR: 104. Regular and normal rate. RR: 18. Regular and unlabored. O2 saturation: 96% on room air. Temperature: 98.4 F (oral). Pain level now 0/10.

Glasgow Coma Scale: 15 - eyes open - spontaneous (4); best verbal response - oriented (5); best motor response - obeys commands (6). -- 17:04 07/17/26 EDT Lusha Fowler, R.N.

17:04 07/17/26. Condition at departure: stable. -- 17:04 07/17/26 EDT Lusha Fowler, R.N.

Departure time: 17:06 07/17/2026. -- 17:06 07/17/26 EDT Lusha Fowler, R.N.

(Electronically signed by Lusha Fowler, R.N. 07/17/26 17:07:32 EDT)

Generated by CorroHealth

Superbill

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

07/17/2026 08:14:00 EDT

Patient: DEWITTE,
ANNE

MRN#:
A069250

Acct#:
557808

Sex:
Female

DOB:
07/16/1952

Age:
74y

Category	Date/Time	Item Description	Facility Code	Professional Code	Quantity	Fee	Total
Nurse/IV/IM/Infusions	07/17/2026 09:49	Hydration additional hour (96361)	1400085		2	\$0.00	\$0.00
Nurse/IV/IM/Infusions	07/17/2026 11:29	Drip/IVPB initial (96365)	1400081		1	\$0.00	\$0.00
Nurse/IV/IM/Infusions	07/17/2026 12:05	IVP additional push (96375)	1490793		2	\$0.00	\$0.00
Nurse/E/M	07/17/2026 17:07	EMERGENCY DEPT VISIT HIGH SEVERITY&THREAT FUNCJ (99285-25)	9928525		1	\$0.00	\$0.00
						Grand Total	\$0.00

Providers

Michael Oates, M.D.

Chief Complaint

PATIENT COMPLAINS OF ELEVATED BLOOD PRESSURE AFTER TAKING SEIZURE MEDICATION. BLOOD PRESSURE ELEVATED.

Principal Diagnosis

ELEVATED TROPONIN

HYPERTENSION

URINARY TRACT INFECTION.

Order Sheet

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

07/17/2026 08:14:00 EDT

Patient: DEWITTE, ANNE MRN#: A069250 Acct#: 557808 Sex: Female DOB: 07/16/1952 Age: 74y

MEASUREMENTS: Wt: 67.4 kg

ALLERGIES: Keppra

MEDICATION/IV/DRIP/FLUID ORDERS

Order Description	Priority	Entered	Acknowledged	Completed
IV NS 500 mL at 250 mL/hr		09:21 07/17/2026 Michael Oates, M.D.		09:49 07/17/2026 Lusha Fowler, R.N.
Rocephin IVPB 1g/100ml NS 1 g diluted in sodium chloride IVPB 0.9 % (minibag plus) 100 mL		11:04 07/17/2026 Michael Oates, M.D.		11:29 07/17/2026 Lusha Fowler, R.N.
Indication: bacterial urinary tract infection				
Reason for ordering with alerts: Clinical consideration given --11:04 07/17/2026 Michael Oates, M.D.				
Tylenol PO 650 mg (NOW)		11:05 07/17/2026 Michael Oates, M.D.		11:28 07/17/2026 Lusha Fowler, R.N.
Indication: back pain				
HydrALAZINE IVP 20 mg		11:49 07/17/2026 Michael Oates, M.D.		12:05 07/17/2026 Lusha Fowler, R.N.
Indication: hypertension				
IV NS Bolus 1000 mL		12:54 07/17/2026 Michael Oates, M.D.	13:00 07/17/2026 Shalie Shiver, L.P.N.	13:01 07/17/2026 Shalie Shiver, L.P.N.
Reason for ordering with alerts: Clinical consideration given --12:54 07/17/2026 Michael Oates, M.D.				
KetorOLAC IVP 15 mg		14:38 07/17/2026 Michael Oates, M.D.		14:56 07/17/2026 Shalie Shiver, L.P.N.
Indication: severe pain				
Reason for ordering with alerts: Clinical consideration given --14:38 07/17/2026 Michael Oates, M.D.				
Aspirin PO 325 mg		15:51 07/17/2026 Michael Oates, M.D.		15:54 07/17/2026 Lusha Fowler, R.N.
Reason for ordering with alerts: Clinical consideration given --15:51 07/17/2026 Michael Oates, M.D.				

LAB ORDERS

Order Description	Priority	Entered	Acknowledged	Collected	Completed
CBC Automated Stat	Stat	09:21 07/17/2026 Michael Oates, M.D.			09:45 07/17/2026 Lusha Fowler, R.N.
Urinalysis w/ Reflex To Culture Stat	Stat	09:21 07/17/2026 Michael Oates, M.D.			10:00 07/17/2026 Lusha Fowler, R.N.
Urine Drug Screen Stat	Stat	09:21 07/17/2026 Michael Oates, M.D.			10:00 07/17/2026 Lusha Fowler, R.N.
CMP Stat	Stat	09:21 07/17/2026 Michael Oates, M.D.			09:45 07/17/2026 Lusha Fowler, R.N.
Magnesium Stat	Stat	09:21 07/17/2026 Michael Oates, M.D.			09:45 07/17/2026 Lusha Fowler, R.N.

Order Description	Priority	Entered	Acknowledged	Collected	Completed
High Sensitive Troponin Stat	Stat	12:55 07/17/2026 Michael Oates, M.D.			10:48 07/17/2026 Lusha Fowler, R.N.
High Sensitive Troponin Stat	Stat	14:41 07/17/2026 Michael Oates, M.D.	14:44 07/17/2026 Shalie Shiver, L.P.N.		14:57 07/17/2026 Shalie Shiver, L.P.N.

DIAGNOSTIC STUDY ORDERS

Order Description	Priority	Entered	Acknowledged	Completed
CT Head wo IV Cont Stat	Stat	13:47 07/17/2026 Michael Oates, M.D.		14:18 07/17/2026 Lusha Fowler, R.N.

Reason for Study: Head Pain

Pregnancy Status: No

Chest 1V Port Stat	Stat	16:46 07/17/2026 Michael Oates, M.D.		16:51 07/17/2026 Lusha Fowler, R.N.
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Reason for Study: Congestion

Pregnancy Status: No

STAFF ORDERS

Order Description	Priority	Entered	Acknowledged	Collected	Completed
EKG Stat	Stat	09:21 07/17/2026 Michael Oates, M.D.			09:25 07/17/2026 Lusha Fowler, R.N.
Cardiac Monitor		09:21 07/17/2026 Michael Oates, M.D.			11:00 07/17/2026 Lusha Fowler, R.N.

[Electronically signed by Michael Oates, M.D. (07/17/2026 09:21 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 11:04 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 11:05 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 11:50 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 12:55 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 12:55 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 12:56 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 12:56 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 13:47 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 14:38 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 14:43 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 15:52 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 16:46 EDT)]

[Electronically signed by Michael Oates, M.D. (07/17/2026 17:08 EDT)]

Visit Overview

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

07/17/2026 08:14:00 EDT

Patient: DEWITTE,
ANNE

MRN#:
A069250

Acct#:
557808

Sex:
Female

DOB:
07/16/1952

Age:
74y

07/17/2026 05:08 PM EDT

ED Arrival: 08:14 07/17/2026 EDT	Pregnancy Status: not pregnant	Recent Travel: no
Language: ENGLISH	Adv Directive: No	Isolation Status:
Ethnicity: Not Hispanic or Latino	Fall Risk:	Infectious Disease Exposure: no
Measurements: 148.6 lb / 67.4 kg	Self-Harm Status: risk	Sepsis Screen:
Chief Complaint: checked by patient, home, (169 / 112), (ELEVATED BP), (SPOUSE C/O PT'STAYED IN BED MOST OF LAST 2 DAYS, AND JUST SAT THERE STARED,AND THAT SHE HAS BEEN UNABLE TO COMPLETE SENTENCES SINCE SHE BEGAN CARBAMAZEPINE), and (SPOUSE STATES IS FROM DR. EASTERLING PUTTING PT ON CARBAMAZEPINE 5 DAYS AGO)		

ALLERGIES

- Keppra

HOME MEDICATIONS

- carBAMazepine 200 mg tablet: 200 mg every 12 hours .
- fosinopril 40 mg tablet: 40 mg once a day .

PAST MEDICAL HISTORY / PROBLEMS

- Hypertension
- LNMP: The patient has had a hysterectomy
- See nurses notes
- Seizure Disorder

PAST SURGICAL HISTORY

- Hysterectomy
- Knee Surgery. RIGHT KNEE

SOCIAL HISTORY

- Nutritional assessment: No deficits
- Functional assessment: No impairments
- Learning needs: No barriers
- Smoking status: No

ED COURSE

MEDICATIONS GIVEN IN EMERGENCY DEPARTMENT

09:49 07/17/26	IV NS 500 mL 250 mL/hr
11:28 07/17/26	Tylenol PO 650 mg
	Rocephin IVPB 1g/100ml NS 1 g diluted in sodium chloride IVPB
11:29 07/17/26	0.9 % (minibag plus) 100 mL 200 mL/hr
12:05 07/17/26	HydrALAZINE IVP 20 mg
13:00 07/17/26	IV NS 1000 mL
14:56 07/17/26	KetorOLAC IVP 15 mg
15:53 07/17/26	Aspirin PO 325 mg

IV SITE INFORMATION

09:45 07/17/26 Site #1 right AC, 20g. Saline lock.

INTAKE

OUTPUT

REASSESSMENT (most recent)

- 08:15 07/17/26. Ambulatory to room. GENERAL / NEURO / PSYCH: Alert. Oriented X 4. Slow gait. Ambulates using a cane. HEENT: Pupils equal, round and reactive to light. No facial asymmetry noted. RESPIRATORY: Respirations not labored. Breath sounds within normal limits. CVS: Capillary refill less than 2 seconds. Pulses within normal limits. GI / GU: Abdomen soft. SKIN: Skin intact. Skin is warm and dry. Normal skin turgor.

VITAL SIGNS

	First Vitals		Last Vitals		
Temp	08:20 07/17/26	98.5 F	17:00 07/17/26	98.4 F	
BP	08:20 07/17/26	183/92	17:00 07/17/26	175/83	
HR	08:20 07/17/26	60	17:00 07/17/26	104	
RR	08:20 07/17/26	16	17:00 07/17/26	18	
O2 Sat	08:20 07/17/26	96% RA	17:00 07/17/26	96% RA	
Pain	08:20 07/17/26	0	17:00 07/17/26	0	
ETCO2	08:20 07/17/26		17:00 07/17/26		
GCS	08:20 07/17/26	15	17:00 07/17/26	15	
RTS	08:20 07/17/26		17:00 07/17/26		

PROCEDURES

NURSING INTERVENTIONS

LABS / STUDIES

LABS / STUDIES ORDERED

- CBC Automated
- Chest 1V Port
- CMP
- CT Head wo IV Cont
- High Sensitive Troponin
- High Sensitive Troponin
- Magnesium
- Urinalysis w/ Reflex To Culture
- Urine Drug Screen

CLINICAL IMPRESSION

Facility Coding Summary

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

07/17/2026 08:14:00 EDT

Patient: DEWITTE, ANNE	MRN#: A069250	Acct#: 557808	Sex: Female	DOB: 07/16/1952	Age: 74y
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Providers: Michael Oates, M.D.

DIAGNOSTIC WORKUP

Chief Complaint

BLOOD PRESSURE ELEVATED. -- Michael Oates, M.D.
PATIENT COMPLAINS OF ELEVATED BLOOD PRESSURE AFTER TAKING SEIZURE MEDICATION. -- Michael Oates, M.D.

Principal Diagnosis

ICD-10 Codes

PROCEDURES

Procedures from Providers:

Procedures from Nurses/Facility:

- EKG (CPT: 93005-76)
- EKG (CPT: 93005)
- IV Infusion Rocephin IVPB 1g/100ml NS (CPT: 96365)
- IV Push HydrALAZINE IVP (CPT: 96375)
- IV Push KetorOLAC IVP (CPT: 96375)
- IV Hydration (CPT: 96361 X2)

SUPPLIES

E&M LEVEL

99285-25

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Milestone Report

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

07/17/2026 08:14:00 EDT

Patient: DEWITTE, ANNE	MRN#: A069250	Acct#: 557808	Sex: Female	DOB: 07/16/1952	Age: 74y
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Milestones	Time	Date	Elapsed Time
Arrival	08:14	07/17/2026	00:00
Triage	08:15	07/17/2026	00:01
To Treatment Room	08:15	07/17/2026	00:01
Seen by Provider	09:09	07/17/2026	00:55
Disposition Decision	16:30	07/17/2026	08:16
Departure	17:06	07/17/2026	08:52